

Summit Orthopedic Institute — Joint Replacement Program

SURGICAL DISCHARGE SUMMARY

SYNTHETIC DOCUMENT — FOR DEMONSTRATION ONLY — CONTAINS NO REAL PATIENT DATA

Patient: Margaret A. Caldwell	DOB: 09/22/1957	MRN: 3047281
Date: October 16, 2024	Provider: Dr. James H. Weston, MD — Orthopedic Surgery	

PROCEDURE SUMMARY

Procedure:	Right Total Knee Arthroplasty (TKA)	Indication:	Severe tricompartmental osteoarthritis, right knee (M17.11)
Date of Surgery:	October 14, 2024	Anesthesia:	Spinal with sedation, adductor canal nerve block
EBL:	Approximately 150 mL	LOS:	2 days (discharged to home)

HOSPITAL COURSE

Ms. Caldwell, a 67-year-old female, underwent elective right total knee arthroplasty for end-stage tricompartmental osteoarthritis with significant functional limitation despite conservative management including physical therapy, intra-articular injections, and NSAIDs. Preoperative radiographs demonstrated severe medial and lateral joint space narrowing with varus deformity.

Intraoperatively, a cemented posterior-stabilized implant system was utilized (Stryker Triathlon, femur size 5, tibia size 4, 10mm PE insert). Patella was resurfaced. Intraoperative range of motion achieved was 0-125 degrees. Wound closed in layers over a drain, which was removed on POD1 with 40 mL output.

Physical and occupational therapy were initiated on postoperative day 1. Patient demonstrated ambulation with a walker, stair negotiation, and independence with home exercise program prior to discharge. DVT prophylaxis was provided with enoxaparin and sequential compression devices. No complications were noted.

WOUND CARE AND ACTIVITY

Wound: Keep incision clean and dry. No submerging in water until cleared by surgeon (typically 3 weeks). Staples will be removed at 2-week follow-up visit. Watch for signs of infection: increasing redness, warmth, swelling, purulent drainage, fever >101.5°F, or wound separation.

Weight-bearing: Full weight bearing as tolerated immediately with front-wheeled walker. Progress to cane when comfortable (typically 2-4 weeks). Avoid kneeling, squatting, or pivoting on the operative knee for 6 weeks.

Exercises: Perform home exercise program 3x daily: ankle pumps, quad sets, short arc quads, straight leg raises, heel slides. Ice knee 20 minutes several times daily for swelling and pain.

DISCHARGE MEDICATIONS

Enoxaparin (Lovenox):	40mg subcutaneous daily x 10 days — DVT prevention	Oxycodone:	5mg PO q4-6h PRN severe pain — taper as tolerated
Meloxicam:	15mg PO daily with food x 4 weeks — take with opioid or alone	Acetaminophen:	1000mg PO q8h scheduled x 2 weeks
Omeprazole:	20mg PO daily — GI protection while on meloxicam		

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